

Jiahang Zhang Entry #1: Anti-Racism in Public Health Act of 2023

The Anti Racism in Public Health Act of 2023 is a law promotion come up by Pressley, Warren and Lee, which is currently in the stage of “Introduced to House”. It is worth mentioning that this is their second time trying to introduce the law and make it applicable.

The Anti-Racism in Public Health Act of 2023 functions as an overall macro level of law, in which the legislation seeks to “declare racism a public health crisis” (Eldahshoury, 2023). By announcing there is a national crisis regarding racial inequality, correlated agencies such as National Center of Anti-Racism as well as relevant laws will better be established in the future.

Congresswoman Lee explained the motions behind the legislation proposal when she was interviewed. She said that the colored groups are experiencing severe disparities on aspects from “asthma to maternal mortality to police brutality” (Eldahshoury, 2023). She also emphasized that the reason why the legislation alarms a nation-wide crisis is because the accumulation of structural racial inequality has existed in the American medical health system long ago, from the founding of this nation. This would effectively prove that such racial problems in the public health system are historical issues that are worth solving.

The background of “re-introducing” the legislation is the result of the nearly half a million federal grants to Boston as the first-of-its-kind federal allowance in alleviating the racial inequalities in the public health system in Boston. Another congresswoman Pressley being the main proposer of the Anti-Racism in Public Health

Act of 2023 was together with Boston Mayor Wu in a celebrating meeting, and after witnessing the initial success of getting public and federal attention on the issues, it can be inferred that she was inspired by it and motivated to promote such motions to a higher level.

Looking through the inner details of this Anti-Racism in Public Health Act of 2023, two things should be noticed and inspiring to analyze when duties of National Center of Anti-Racism are introduced. The first is that the center should utilize new knowledge and integrated resources to identify the general mechanisms behind the racism in the Public Health System (Pressly, 2023). Different from other bureaucratic abstract duties, identifying the causational factors behind phenomena is significant in solving the problems. Whether it is the financial incapability or understanding gaps on receiving medical services or others, only by finding the reasons could the U.S. government find targeted methods to better solve the problem, which makes the act a landmark.

Another emphasis point is that in the act, “a national and global conversation regarding the impacts of racism on the health” will be enacted by the center (Pressly, 2023). Such an international program cannot be constructed without a large integration of overall resources, which will fill the previous blank space in such an area. Many countries in the world do not suffer from racial challenges in their medical system as harsh as America does today, and by observing their well-constructed sample (such as well-being and medical insurance in North Europe countries),

experience and references can be recorded and copied, which will help improve American today's chaotic circumstances.

The two advantages brought by a more formal construction of a national-wide center in preventing racial inequalities is necessary, which none of the described benefits can be achieved if the act is turned down, at least based on the current efforts and statuses functioning in the United States.

Pressley, A. (n.d.). The anti-racism in public health act of 2023 - ayanna pressley.

<https://pressley.house.gov/wp-content/uploads/2023/04/Anti-Racism-in-Public-Health-Act-2023-One-Pager.pdf>

Eldahshoury, M. (2023, May 3). *Pressley, Warren, Lee re-introduce anti-racism in public health act*. Ayanna Pressley.

[https://pressley.house.gov/2023/04/26/pressley-warren-lee-re-introduce-anti-racism-in-public-health-](https://pressley.house.gov/2023/04/26/pressley-warren-lee-re-introduce-anti-racism-in-public-health-act/#:~:text=WASHINGTON%20%E2%80%93%20Today%2C%20Congresswoman%20Ayanna%20Pressley,Racism%2C%20and%20establish%20a%20Law)

[act/#:~:text=WASHINGTON%20%E2%80%93%20Today%2C%20Congresswoman%20Ayanna%20Pressley,Racism%2C%20and%20establish%20a%20Law](https://pressley.house.gov/2023/04/26/pressley-warren-lee-re-introduce-anti-racism-in-public-health-act/#:~:text=WASHINGTON%20%E2%80%93%20Today%2C%20Congresswoman%20Ayanna%20Pressley,Racism%2C%20and%20establish%20a%20Law)

Jiahang Zhang Entry #2: "CARES" Act

The CARES Act, enacted in 2020 in response to the COVID-19 pandemic, was an important piece of legislation that solved not only the economic and health system problems caused by the crisis, but also targeted to confront an eternal topic related to the issue of racial inequality in the U.S. system. In the Act, 2.2 trillion of dollars are issued as stimulus after passing through congress and signed by the temporary president, Donald Trump, who has long been recognized as a tough figure on minority immigrants from his previous attitude and policies.

While offering direct payments to individuals and financial subsidy to businesses, the CARES Act also realized the unequally proportional influence on colored minority communities by pandemic. To be more specific, colored communities, with in fact many legacies of infrastructures are not well-constructed, suffered much more severe consequences during the pandemic, represented by their caught affection rates, medication covering rates, total curing cases number and so on (Yearby, 2020). Considerable resources are distributed into the healthcare sector, enhancing the development of many hospitals and COVID-19 testing offices. This was significant in alleviating the racial inequalities in the healthcare area, as minority communities were disproportionately impacted by the virus.

Furthermore, the act extended unemployment benefits, helping those who lost their jobs, a demographic that included mostly a significant number of marginalized groups. This shows the firm and solid step made by the act in promoting equitable outcomes despite the existing disparities among racial differences. This focus also strengthened access to healthcare services, admitting that underserved communities with colored skin are indeed facing barriers to receive them.

In a journal analyzing the minority groups and racial disparities during the COVID-19, according to the author Yearby and Mohapatra, The Hill-Burton Act used the tax money of racial and ethnic minorities to build healthcare facilities that provided care to whites but barred racial and ethnic minorities from receiving care (Yearby, 2020). This makes the chances of catching respiratory diseases increase significantly among African American due to their much fewer access for medicines and medical treatment, turning eventually into a higher risk of catching and suffering from the diseases.

Moreover, in the journal, Yearby confirmed the limited influence of CARES Act, as well as two other acts, Patients Protection and ACA, on the racial problems in the health system. She mentioned that despite the "general level" of distribution of massive amounts of federal funds in helping the dilemma, the reality that there are limited chances for African Americans to receive COVID-19 treatment and other health conditions services has fundamentally remained, which means the problems still exist. As Sager and Socolar stated in their research, after taking surveys among several closures of African American communities, they surprisingly found that

starting from 1980 to 2010, the hospital closing rates and physicians who left the neighborhoods are both increasing, and this trend sustains till recent years, which further prove the limited access for African American citizens (Yearby, 2020).

Of course, it is not their only choice to go to hospitals in their own neighborhoods for African Americans. However, if considering what the situations were like during the pandemic and the cost of hospitals in America, especially those with more expensive fees in white neighborhoods, it is obvious to figure out the catastrophic impact on black communities living under a fact that their medical coverage is threatened and cannot be persisted.

Reference:

Yearby, R., & Mohapatra, S. (2020). Law, structural racism, and the COVID-19 pandemic. *Journal of Law and the Biosciences*, 7(1), lsaa036.

Yash Patil Entry 1: The Patient Protection and Affordable Care Act Section

1557

The Patient Protection and Affordable Care Act was issued by the Department of Health and Human Services. This Act was first issued on March 23, 2010, with the idea to add more to as time goes on. At first, it was implemented so healthcare and insurance for healthcare would be convenient and easier to be comprehended by the public. It was also important as it made healthcare more affordable, especially for lower income patients. They issued Section 1557 in 2016 which states that it is against the law for the discrimination of race, color, ethnicity, country of origin, sex, or age in healthcare programs that are funded by the government. This also includes all forms of government issued insurance as well.

The idea of the Patient Protection and Affordable Care Act was to make healthcare programs more affordable, but it was noticed that discrimination was prevalent. The National Health Law program helped develop Section 1557 as a means to combat this issue. It was continued to be worked on after President Obama passed the Affordable Care Act. The program wanted to see a change in the way healthcare handled those situations involving discrimination and hoped that the protection that were to be put in place could make a change in that regard.

The hopes were that if Section 1557 was passed and implemented properly, the lives and experiences of colored people would be improved as they would have better unbiased healthcare and insurance coverage as before they lacked that.

Another major aid in Section 1557 was their implementation of protections for non-english people, especially for those with different ethnicities and races that spoke different languages. Where as before, they would be unable to have access at that facility, they can now be able to use that option. This is mainly because federally supported health care cannot turn down patients due to a language barrier and now are required to provide either language interpreting assistants or translated documents. This allows for people of different race and ethnicity to be able to access the same resources as others that do not face that issue of discrimination.

Additionally, the Biden Administration plans to improve and strengthen Section 1557, and make the protections stronger which should improve the lives of millions of people in the United States.

Overall, the Patient Protection and Affordable Care Act was designed to improve the experience patients would have with healthcare systems, especially those in lower income households. This allowed for many people who could not afford good healthcare to be able to access it. This left an issue though, that discrimination was still a factor. People of color and other minorities were experiencing harder times with getting the healthcare they deserved so Section 1557 of the ACA was passed in hopes to solve the issues that were in place. This tremendously helped those of color to receive the healthcare they needed and allowed for people of other races to be not only understood, but also taken care of instead of being turned away.

Citations

Ziama, G. (2022, August 24). What is section 1557? an introduction. National Health Law Program. <https://healthlaw.org/what-is-section-1557-an-introduction/>

Patient protection and affordable care act . Glossary | HealthCare.gov. (n.d.).
<https://www.healthcare.gov/glossary/patient-protection-and-affordable-care-act/>

HHS issues proposed rule under section 1557 of the Affordable Care Act:

Nondiscrimination in health programs and activities. McDermott Will & Emery. (2023, March 8). <https://www.mwe.com/insights/hhs-issues-proposed-rule-under-section-1557-of-the-affordable-care-act-nondiscrimination-in-health-programs-and-activities/>

Yash Patil Entry 2: Health Equity and Access under Law (HEAL)

The Health Equity and Access under Law for Immigrant Families Act was a Bill passed in 2023. It was passed to help those families that are of different races and the aid they needed in the healthcare field. There were many examples of immigrants that were not given the same resources and healthcare opportunities due to their race and that they were immigrants. It was such a difficulty for them as it takes a 5 year period where they must wait for Medicare related benefits as they have to be lawful immigrants. The bill gives the immigrants the ability to receive Medicaid without the 5 year waiting period and without the documentations needed.

Immigrants do not have the citizenship that native US born people have and this comes as a detriment to their health care. This leads to many issues related to lack of healthcare insurance which leads to more severe financial and and patient health

issues. Representative Jayapal says that all humans have the right to healthcare, regardless of their race and whether they are from the United States or not. Due to this, their goal was to eliminate barriers that Non-Americans face in American healthcare. This was a great benefit and helped with the racial discrimination of other barriers that were also in place in healthcare.

The goal of this was to provide families of color and those who do not have citizenship due to immigrant status a way to have access to means of providing healthcare in an easier and more friendly way. This would include healthcare and insurance. Where before, immigrants and families of color would be turned away from the services of many healthcare and insurance facilities, they now have that access they needed and deserve.

Section 2a9 of the bill states that immigrants, especially those of color like black, asian, islander, latin, and indigenous, are the people that are harmed the most by the American healthcare system. The difficulty also lies within help and care with pregnancy and childbirth. The bill also states that black people are four times as likely to experience a pregnancy related death than that compared to white people as they were not given the proper healthcare options or priorities. This is an issue with human rights and this bill was passed to try and negate more of these situations from occurring.

The Bill also states that its purpose is to make sure all people, citizens and immigrants, have access to health care so it can remove the harmful racial, gender,

and economical issues that are prevalent and give the patients the necessary healthcare for an affordable price.

In conclusion, the Health Equity and Access under Law for Immigrant Families Act was a bill passed to improve the health care experience of immigrants and colored immigrants. This was able to reduce the amount of obstacles and barriers that were placed for patients to receive proper health care and insurance. The goal of the Bill was to make sure immigrants can receive the same treatments as all humans should have access to healthcare as a right, and it should not matter about their country of origin or their race that they receive or do not receive healthcare.

Citations

H.R.5008 - 118th Congress (2023-2024): Health Equity and access under ... (n.d.).

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Jeske, S. (2023, July 27). *Jayapal, Booker, and Barragán introduce legislation to lift barriers to health care for immigrants*. Congresswoman Pramila Jayapal.

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Emma Wisniewski Entry #1: Hospital and Survey Construction Act (Hill-Burton Act)

During the 1900s, the United States was a very segregated country. Different races and ethnicities had assigned bathrooms or water fountains they were directed to use. This created extremely negative views and opinions surrounding people of color.

This idea of segregation translated heavily in the 1960s. Hospital discrimination was at an all time high and it became a widespread issue across the United States. Certain ethnic groups were denied care and denied even admission into medical school simply based on their skin color. There was also a shortage of hospital beds in the United States during this time as well. This made it easier for people to discriminate in the healthcare systems by ignoring people of color. Because the views at this time were very negative towards those of color, healthcare staff and professionals thought it was acceptable to treat others this way. This caused many people who were being discriminated against to become even more upset with the unfair treatment. Public health officials, medical staff, and the federal government decided it was time to make a change. These groups began working together to try and create a solution to eliminating, or at least lessening, discrimination in the healthcare systems.

These efforts ultimately are what helped to pass the Hospital and Survey Construction Act. This act was passed in August of 1965 as a result of hard work from many different people. This law gave hospitals, nursing homes, and many other healthcare facilities money through grants and loans to update the buildings. This

meant that hospitals could obtain more beds and build additional rooms. It also created a new policy that all healthcare professionals should follow called the “separate, but equal” treatment.

Conrad Odell Pearson, Harold Burton, and Lister Hill were three of the most important figures associated with the Hospital and Survey Construction Act. Harold Burton was a republican United States Senator from Ohio and he represented the northern part of the country. Lister Hill was democratic United States Senator from Alabama and he represented the southern part of the country. Aforementioned, the country was divided between people. The joining of these two senators from the north and south to pass this act was influential for many people. It showed that people should not be afraid of each other and should rather try to work together to accomplish more. Conrad Odell Pearson held hospitals and healthcare facilities responsible for their acts of discrimination. He got to the root of the problems in those specific buildings and made sure any problems were taken care of. He also fought the government on many levels and educated people about the impact of discrimination in healthcare.

This law, along with many other court cases, became a turning point for attempts in trying to eliminate discrimination in healthcare. Pearson, Burton, and Hill set a strong foundation for people trying to make a difference within healthcare. The act itself helped to limit discrimination in healthcare systems, but there is still a long way to go in completely eliminating discrimination in healthcare.

Reynolds, P. P. (2004, May). *Professional and Hospital Discrimination and the US Court of Appeals Fourth Circuit 1956-1967*. American journal of public health. <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC1448322/>.

Emma Wisniewski Entry #2: Charlotte Woodward Organ Transplant

Discrimination Prevention Act

Discrimination in healthcare systems is an ongoing issue within the world despite many efforts to eliminate it. This discrimination also does not only occur with race or ethnicity, but with health conditions as well. The field of medicine is very large and complex, so people can get away with discrimination more easily, which is not acceptable.

There is a lot of discrimination within the organ transplant system of healthcare. Receiving an organ can be life-changing for people, but not everyone who needs one gets this chance. Potential recipients are often added to a list and those in the most dire need are at the top. If an organ donor is matched with a potential recipient, the actual recipient is determined at the discretion of the medical staff at that time. This leaves a lot of room for discrimination to occur. Oftentimes, healthcare professionals input their biases and opinions into these decisions and this can severely affect the outcome of another's life.

People with disabilities often face discrimination in health care. A lot of people have a belief that people with disabilities are less than them and treat them as such.

However, just because a person has a disability does not mean they deserve any less of treatment by medical professionals.

Charlotte Woodward is a girl with Down Syndrome who is fighting to eliminate discrimination in healthcare. Many years ago she received a life-saving heart transplant after waiting for a long time. Now, she is advocating for this with disabilities to ensure they receive equal treatment. This act was presented in April of 2023. The bill is aimed at protecting many different rights associated with receiving an organ donation if you are disabled. It ensures that people with disabilities are not denied an organ transplant because of their disability, that they are not refused access on the list to receive one, or that insurance coverage is not denied for one. This bill also ensures that people with disabilities who receive an organ transplant are provided with the necessary post-operative care. There are many other rules and regulations covered by this bill, but those are the main ideas. The organ transplant process is very complicated.

One of the creators of the bill, United States Representative Kat Cammack, had a first-hand experience of a baby passing away due to being denied an organ transplant. This infant was born with Down Syndrome and was denied a transplant because of his disability. The healthcare professionals' deemed this baby to not be worth the transplant procedure which ultimately led to his death. People often have judgements about people with disabilities and their quality of life, but form these opinions on false pretenses.

Because this law was presented just this year, it has not progressed farther than being introduced to congress. However, this bill shows that discrimination is still an ever present issue within healthcare systems. Although there have been many efforts made by a range of people to eliminate discrimination in healthcare, there are still a lot of other problems. People are still actively working to eliminate discrimination in healthcare.

Reps. Kat Cammack & Debbie Dingell introduce bipartisan Charlotte Woodward Organ Transplant Discrimination Prevention Act. Representative Kat Cammack. (2023, April 19). <https://cammack.house.gov/media/press-releases/rep-kat-cammack-debbie-dingell-introduce-bipartisan-charlotte-woodward-organ>.

Yasmeen **The Allied Health Workforce Diversity Act (H.R. 3320/S. 1679) -**

Saffer , A. (2023, January 6). *End of year legislation creates new allied health workforce diversity program.* Aota.org. <https://www.aota.org/advocacy/advocacy-news/2022/end-of-year-legislation-creates-allied-health-workforce-diversity-program>

Sen. Lisa Murkowski. (n.d.). *Office of senator Bob Casey FY24 appropriations guidance.* Allied Health Workforce Diversity Act.

https://www.casey.senate.gov/imo/media/doc/fy24_guidance_for_reference_final.pdf

The Allied Health Workforce Diversity Act was signed in December of 2022, introduced in 2019, and this act established the idea that the healthcare workforce in the United States was going to become more diverse in terms of the employers as well as opening up access to health care to everyone. This act was put in place with the intention of including diversity in the healthcare system to therefore strengthen the healthcare system.

The benefit of having a healthcare workforce who is representative of the population in terms of diversity is that the overall quality of health care will improve. Additionally, cultivating an inclusive workforce is essential in order to increase accessibility to health care for all people in the U.S. The goal of this act is to represent racial and ethnic groups who are underrepresented in the healthcare field, but also to help the patients of different racial groups to ensure they have access to health care, despite their gender and ethnicity. Also, it's important to note that this act also has the desire to focus on the communities that have faced healthcare disparities in the past to therefore improve health care services to those communities as well.

There has been a study that reinforces the need for this act, and the study mentions how patients tend to receive better outcomes if they receive treatment from a healthcare provider from their own cultural background. This is an interesting finding from this study because this can relate to one of the goals of the act which is not only grant health care access to different racial and ethnic groups, but also obtain employers from different backgrounds in the U.S. One way to follow up with this plan is that the act has granted specific programs to promote diversity in health care environments. The grant programs that this act has established support educational institutions, organizations, as well as supporting health care providers in order to enhance diversity within people who work in any health related environment, specifically, health care workers.

This act also recognizes how collecting data and reporting it is crucial in order to track progress with identifying if the changes being made succeeded, failed, or maybe didn't make a difference. The type of data that the act has stated that is to be collected is a demographic data collection of ethnic, and racial groups, as well as taking gender into consideration. It is important to collect data to allow policymakers to review the steps taken to include diversity into the health care system in ways to improve access to health care as well as quality and determine if it's effective or not.

One last key takeaway from this act is that they would create a \$5 million a year grant in order to provide grants for students who have diverse backgrounds and this would allow them to be in programs related to the health field they are interested in. For example, a program preparing to be a physical therapist or an occupational therapist. Not only does this assist those who have diverse backgrounds and are interested in working in the healthcare field, but this is also related to the idea that patients of different racial and ethnic groups have the opportunity of being taken care of by someone of their own culture.

Equal Health Care for All Act

Adam Schiff. (2020, September 29). *Schiff introduces legislation addressing systemic racism and inequality in health care by treating it as a civil rights issue*. Home.

https://schiff.house.gov/news/press-releases/schiff-introduces-legislation-addressing-systemic-racism-and-inequality_in-health-care-by-treating-it-as-a-civil-rights-issue

The Equal Health Care for All Act was introduced to recognize systemic racism and inequalities present in the health care system in the United States. Although it has not been signed yet, there are many benefits to this act. The main goal of this legislation is to analyze and investigate complaints coming from patients in regards to their experiences with discrimination in relation to health care. Additionally, the main reason for establishing this act is to create a universal health care system in which no one is treated differently just because they have a different racial or ethnic background. In other words, every citizen in the U.S, under this system, is able to access healthcare services they need regardless of their race, employment and income status. This ultimately ensures that having access to health care isn't a privilege and everyone should be guaranteed the same rights.

There are many benefits to this act that cover medical services such as prescription medication, hospitalization, dental care, and many more services related to health needs. The act having these kinds of benefits is great because everyone receives the same resources no matter what their circumstances are and it can reduce stress for patients if they were having trouble accessing any resources related to health care.

This act also takes price into consideration because healthcare services can be very expensive and many people in the U.S may not be able to afford health care, and source of income shouldn't be a determining factor for whether a person receives the care they need or not. The act put cost control measures in place. This means that patients are able to negotiate prices with health care employers to be able to match their needs based on the money they have and this would be an equal opportunity for everyone. In addition, the overall cost for prescription medicine and necessary health services will decrease making it affordable for patients.

A crucial part of the act is that they emphasize the importance of health for every patient. Due to the fact that each patient will be treated equally despite racial background, this act states that each patient receives the right to have healthcare. With that being said, because of this, there will be an increase in access to primary care and health care services for every person in the U.S and society will be healthier as whole. This could potentially reduce many illnesses, diseases, untreated injuries, etc.

Lastly, this legislation plans to create a Federal Health Equity Commission which would monitor the progress of health equity efforts, and collect data in an efficient and effective way in order to provide more equitable health care practices with more information. As well as providing grants to others to also practice equitable healthcare.

All in all, the Equal Health Care for All Act has the ultimate goal of wanting to end the healthcare crisis in the United States by giving everyone access to the same healthcare services no matter the person's circumstances. The act also was introduced in order to protect access to equitable health care for American citizens.

